

2026 CLINICAL PRACTICE GUIDELINE FOR ATRIAL FIBRILLATION

AHA / ACC / HRS Task Force on Clinical Knowledge & Decision Support

1. INTRODUCTION AND EPIDEMIOLOGY

Atrial fibrillation (AF) is the most common sustained cardiac arrhythmia encountered in clinical practice. The lifetime risk of developing AF is approximately 1 in 3 in individuals of European ancestry. AF is associated with a 5-fold increased risk of ischemic stroke and a 3-fold increased risk of heart failure. Early rhythm control and comprehensive stroke prevention are paramount.

2. DIAGNOSTIC WORKUP

Diagnosis requires documentation of irregular supraventricular rhythm without distinct P waves on a standard 12-lead ECG or rhythm strip persisting for at least 30 seconds. Initial evaluation should include transthoracic echocardiography (TTE), serum thyroid-stimulating hormone (TSH), complete blood count, and renal function panel.

3. STROKE RISK STRATIFICATION

The CHA₂DS₂-VASc score is recommended for assessment of thromboembolic stroke risk in non-valvular AF. Points are assigned for Congestive heart failure (1), Hypertension (1), Age ≥ 75 (2), Diabetes mellitus (1), Prior Stroke or TIA (2), Vascular disease (1), Age 65-74 (1), and Sex category female (1). Anticoagulation is strongly recommended for males with score ≥ 2 and females with score ≥ 3 .

4. FIRST-LINE ANTICOAGULATION

Direct oral anticoagulants (DOACs) are recommended in preference to warfarin due to lower rates of intracranial hemorrhage. Recommended options include Apixaban 5 mg orally twice daily, Dabigatran 150 mg orally twice daily, or Rivaroxaban 20 mg orally once daily with the evening meal.

5. RECOMMENDED ORAL ANTICOAGULANTS IN NON-VALVULAR AF

Drug Name	Standard Dosage	Frequency	Dose Reduction	Renal Elimination
Apixaban	5 mg oral	Twice daily	2.5 mg BID if 2+ criteria	27% Renal
Rivaroxaban	20 mg oral	Once daily	15 mg QD if CrCl < 50	66% Renal
Dabigatran	150 mg oral	Twice daily	75 mg BID if CrCl < 30	80% Renal

6. RATE CONTROL PHARMACOTHERAPY

Beta-blockers (such as Metoprolol succinate 50-200 mg once daily or Bisoprolol 5-10 mg once daily) or non-dihydropyridine calcium channel blockers (such as Diltiazem 120-360 mg once daily) are recommended as first-line agents to control ventricular response in AF patients with preserved LVEF. Digoxin is recommended as a secondary agent when beta-blockers are insufficient or contraindicated.

7. RHYTHM CONTROL AND ABLATION

Antiarrhythmic drugs (such as Flecainide 50-150 mg twice daily in patients without structural heart disease, or Amiodarone 200 mg once daily in patients with heart failure) are effective for sinus rhythm maintenance. Catheter ablation is indicated as first-line therapy in selected young symptomatic patients or when antiarrhythmic pharmacotherapy fails.